

HCA Healthcare

HCA Healthcare, Inc.

NYSE: HCA · Nashville, Tennessee, United States

\$427.71

Closing price, September 18, 2026 (latest completed session) · 52-week range \$353.99 to \$556.52



Executive Summary

Business

HCA Healthcare owns and operates 189 hospitals and approximately 2,600 ambulatory sites of care across 19 U.S. states and England, making it the largest for-profit health system in the United States by hospital count, bed count and net patient revenue. Its revenue engine is fee-for-service acute and outpatient care, paid predominantly by commercial insurers, Medicare and Medicaid, delivered through a decentralized, market-cluster operating model built around three geographic operating groups.

Financial Condition

Revenue grew from \$58.75 billion in FY2021 to \$75.60 billion in FY2025 (7.1 percent year over year), with trailing twelve month (TTM, period ended June 30, 2026) revenue of \$78.01 billion, TTM EBITDA of \$15.75 billion and TTM net income of \$6.84 billion. TTM operating cash flow of \$11.12 billion funded \$5.13 billion of capital expenditure, leaving free cash flow of \$6.00 billion, against a net debt position of \$50.42 billion (Debt/EBITDA of 3.07x) and a book equity position that is negative because of sustained, debt-funded share repurchases.

Market Position

HCA's principal for-profit competitors are Tenet Healthcare (hospitals plus a large ambulatory surgery center platform), Universal Health Services (acute care and the largest U.S. behavioral health operator) and Community Health Systems (a smaller, more leveraged rural and community hospital operator); nonprofit systems such as CommonSpirit Health and Ascension compete in many of the same metropolitan markets. HCA's principal advantage is density-driven scale inside the specific metro markets it chooses to operate in, which supports negotiating leverage with commercial payers and lowers the per-admission cost of capital investment; its principal weakness is direct exposure to U.S. federal health policy, in particular Medicaid supplemental payment programs and the individual insurance exchange subsidies that determine how many of its patients are insured.

Valuation

At the September 18, 2026 close of \$427.71, HCA's market capitalization was \$92.60 billion and its enterprise value \$143.02 billion, implying a trailing P/E of 14.32x (forward 13.96x), EV/EBITDA of 9.08x, EV/Sales of 1.83x and P/FCF of 15.44x. These multiples sit above both Tenet Healthcare (EV/EBITDA 6.64x) and Universal Health Services (EV/EBITDA 5.76x), reflecting HCA's scale, margin stability and market-leading position rather than a discount for the sector's current reimbursement uncertainty.

Investment Framework

WHAT IS ATTRACTIVE

Consistent volume growth, a five-year run of margin expansion into FY2025, an aggressive and historically well-timed capital return program, and the scale advantages of being the largest operator in most of its metro markets.

WHAT IS CONCERNING

A structurally negative book equity position built on debt-funded buybacks, Debt/EBITDA above 3x, and a 2026 payer-mix shock from the expiration of enhanced ACA premium tax credits that has already pushed uninsured admissions sharply higher.

WHAT MUST GO RIGHT

Supplemental Medicaid payment programs and state-level policy actions need to continue offsetting the loss of exchange-subsidized patients, and same-facility volume growth needs to persist at a pace that keeps operating leverage working in HCA's favor.

WHAT COULD BREAK THE THESIS

A sustained rise in uncompensated care that is not offset by supplemental payments, a renewed rise in labor costs, or a tightening in credit markets that raises the cost of refinancing HCA's approximately \$51.6 billion of total debt.

WHAT APPEARS TO BE PRICED IN

A premium multiple to hospital peers consistent with continued mid-single-digit revenue growth and stable-to-improving margins; the market does not appear to be pricing a severe or prolonged payer-mix downturn.

Investment Profile

BUSINESS QUALITY	Large-scale, essential-service operator with a defensible, market-cluster model; earnings quality is high but revenue is concentrated in a single, heavily regulated national payer system.
FINANCIAL QUALITY	Strong and growing operating cash flow supports an unusually leveraged balance sheet; negative book equity is a capital-structure choice, not a sign of operating distress.
COMPETITIVE POSITION	Market leader by scale; advantage is local density in its chosen metro clusters rather than a single national moat.
GROWTH PROFILE	Mid-single-digit organic volume and revenue growth, supplemented by selective bolt-on acquisitions (for example, the September 2026 purchase of the College of Health Care Professions).
BALANCE-SHEET RISK	Elevated; Debt/EBITDA of 3.07x and negative equity leave limited room for a simultaneous earnings and multiple shock.
VALUATION	Premium to hospital-sector peers on EV/EBITDA and P/E; premium is currently justified by scale and margin stability rather than by a valuation discount.
MARKET AND TECHNICAL PROFILE	Trading below its 200-day moving average and roughly 23 percent off its 52-week high, with a beta of 1.11 and an analyst consensus of Buy.

A. About the Business

A1. Business Environment

HCA operates in the U.S. hospital and health system industry, the segment of the broader healthcare delivery value chain that provides acute inpatient care, emergency care, surgery and an increasingly large share of outpatient diagnostic and procedural volume. The industry sits between two much larger groups it does not control: the payers (Medicare, Medicaid and commercial insurers, who set or negotiate reimbursement rates) and the labor and supply markets (nurses, physicians, pharmaceuticals and medical devices, whose costs the operator absorbs). Value is created where an operator can combine sufficient local scale to negotiate favorable commercial rates with high enough acuity and occupancy to spread its largely fixed cost base (buildings, imaging equipment, standby emergency capacity) over more paying admissions; value is destroyed where an operator lacks local scale, depends on a single large employer or payer, or cannot control labor costs.

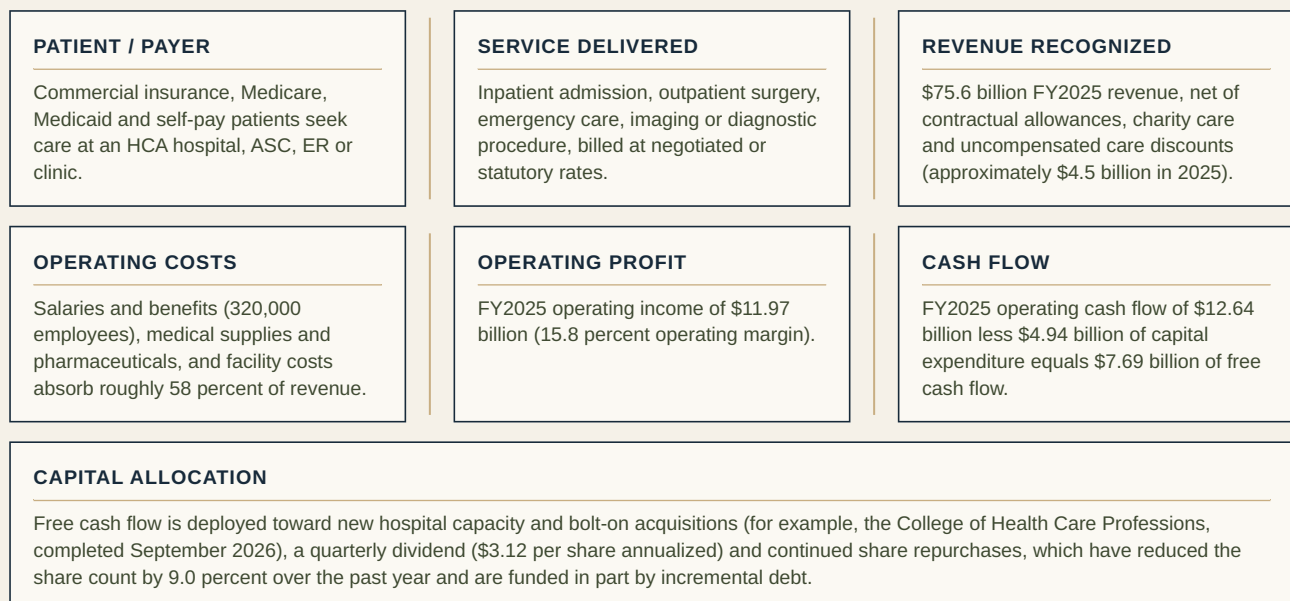
The U.S. hospital industry is best described as mature and consolidating, with a structural, policy-driven transformation underway. Total hospital industry revenue has grown for decades in line with national health expenditure, but reimbursement growth is increasingly disconnected from underlying volume growth, most recently illustrated by the expiration of enhanced Affordable Care Act premium tax credits at the end of 2025, which is expected to cost U.S. hospitals, physicians and other providers more than \$32 billion of revenue and add \$7.7 billion of uncompensated care industrywide in 2026 (Sourced, Urban Institute analysis published by the Robert Wood Johnson Foundation, reported by Fierce Healthcare). At the same time, care is migrating from inpatient beds to ambulatory surgery centers, urgent care and home-based settings, a trend that rewards operators, including HCA, that have built out an ambulatory network alongside their hospitals rather than depending solely on inpatient admissions.

A2. What Does the Company Do?

HCA's core business is operating general acute care hospitals; as of the most recent company disclosure (data as of March 31, 2026) its network comprised 189 hospitals (179 general acute care, seven behavioral health and three rehabilitation hospitals, per the FY2025 Form 10-K), together with roughly 2,600 ambulatory sites, including freestanding emergency departments, urgent care and walk-in clinics, ambulatory surgery centers, imaging and diagnostic centers, physician practices, home health agencies and hospices. Patients are admitted through emergency departments, physician referral or scheduled surgery; HCA is paid predominantly on a fee-for-service basis by Medicare, Medicaid and commercial insurers, with a smaller share of revenue from managed-care capitation and self-pay patients. The company analyzes data from approximately 47 million patient encounters a year across this network (Sourced, HCA Healthcare corporate disclosure).

Reported revenue is organized into three geographic operating groups (National, Atlantic and American) plus a Corporate and Other category, rather than by clinical service line; in FY2025 the American Group generated \$26.45 billion of revenue, the Atlantic Group \$24.71 billion, the National Group \$21.28 billion and Corporate and Other \$3.17 billion (Sourced, company financial disclosures via S&P Global Market Intelligence). The largest operating costs are salaries, wages and employee benefits (HCA employs approximately 320,000 people), followed by supplies (principally pharmaceuticals and medical devices), and depreciation on the hospital real estate and equipment base; capital expenditure was \$4.94 billion in FY2025 and \$5.13 billion on a trailing twelve month basis, reflecting both maintenance capital and continued investment in new capacity in growing metro markets.

Figure A2.1: How a Patient Dollar Moves Through HCA Healthcare



Figures are FY2025 (year ended December 31, 2025) except where marked TTM. Source: HCA Healthcare Form 10-K and corporate disclosures; analyst-constructed flow diagram (Reasoned).

A3. Business and Relationship Network

HCA's ecosystem is built around payers, physicians and a small number of strategic technology and education partners rather than a traditional industrial supply chain. Its most important counterparties are the large commercial payers (UnitedHealthcare, Elevance Health, Cigna and others) with whom it negotiates multi-year network contracts, and the Centers for Medicare and Medicaid Services (CMS), which sets Medicare reimbursement rates and administers the state Medicaid supplemental payment programs that have become an increasingly important offset to commercial payer-mix pressure. Independent physicians who are not HCA employees but who admit patients to HCA facilities are a second critical relationship; HCA invests in physician recruitment, graduate medical education (it operates one of the largest graduate medical education platforms in the country) and, since September 2026, workforce pipeline programs such as the College of Health Care Professions, a nursing and allied-health training operator it acquired to help secure its own staffing needs.

HCA HEALTHCARE - SELECTED ECOSYSTEM RELATIONSHIPS

Category	Counterparty / Example	Why It Matters
Commercial payers	UnitedHealthcare, Elevance Health, Cigna	Source of the highest-margin patient volume; contract rates set unit economics
Government payers	CMS (Medicare, Medicaid)	Sets statutory rates; supplemental payment programs are a 2026 offset to exchange losses
Admitting physicians	Independent and employed medical staff	Physicians, not HCA, control most admission decisions
Clinical technology	MEDITECH (Expanse EHR)	Electronic health record backbone; rollout live across HCA's Capital and North Florida divisions as of September 2026
Workforce pipeline	College of Health Care Professions (acquired Sept. 2026)	Addresses nursing and allied-health staffing, HCA's largest single cost driver
Research and education	HCA Healthcare Research Institute; graduate medical education programs	Supports physician recruitment and clinical outcomes data used in payer negotiations

Source: HCA Healthcare corporate disclosures and Business Wire press releases, 2026 (Sourced).

A4. Competition

HCA's most economically relevant competitors are the other large, multi-state for-profit hospital operators that compete for the same commercial-payer patients, physicians and acquisition targets: Tenet Healthcare, Universal Health Services and Community Health Systems. Tenet competes most directly on scale and on ambulatory surgery, where its USPI platform is a national leader; Universal Health Services competes on acute care but is structurally different in that roughly a third of its business is behavioral health, a segment where HCA has minimal presence (seven behavioral hospitals against HCA's 189-facility network). Community Health Systems is smaller and more financially stretched, illustrating the downside case for a hospital operator without HCA's scale or balance-sheet flexibility. In most individual metro markets, HCA's more immediate competitors are large nonprofit systems, such as CommonSpirit Health and Ascension, which are not exchange-listed and are excluded from the financial comparison below but are referenced qualitatively because they compete directly for the same patients and physicians.

COMPETITIVE COMPARISON - SELECTED U.S. FOR-PROFIT HOSPITAL OPERATORS (TTM DATA AS OF SEPTEMBER 18-21, 2026)

Company	Revenue (TTM)	EBITDA Margin	Market Cap	Model / Differentiator
HCA Healthcare (HCA)	\$78.01B	20.2%	\$92.60B	Largest scale; balanced acute-care and ambulatory network; minimal behavioral health
Tenet Healthcare (THC)	\$21.81B	22.2%	\$21.10B	Large ambulatory surgery center platform (USPI) alongside a smaller hospital base
Universal Health Svcs. (UHS)	\$18.11B	14.9%	\$10.41B	Leading U.S. behavioral health operator plus acute-care hospitals
Community Health Systems (CYH)	\$12.5B (FY2025)	Not sourced in this report	Not sourced in this report	Smaller, more leveraged rural and community hospital operator; 2026 consensus revenue guidance of approximately \$11.6 billion implies a revenue decline

Source: StockAnalysis.com / S&P Global Market Intelligence company statistics pages, accessed September 21, 2026; Community Health Systems figures from Becker's Hospital Review, 2026 net revenue outlook summary (Sourced). EBITDA margin calculated by the analyst as TTM EBITDA divided by TTM revenue (Calculated).

A5. Competitive Differences

HCA's principal advantage is what the company itself describes as a market-density strategy: rather than operating a thin national footprint, it concentrates multiple hospitals, surgery centers and physician practices inside a smaller number of metro areas (for example, Houston, Dallas, Nashville, Denver and South Florida), which gives it enough local bed and specialist capacity to negotiate favorable commercial contracts and to absorb fixed costs, such as trauma-center standby capacity, over a larger patient base than a single-hospital competitor could. This is a structural, scale-based advantage rather than a brand or intellectual-property moat, and it strengthens over time as HCA adds ambulatory capacity around its hospitals; sustained market-share gains reported by management (management states share gains exceeding 28 percent when behavioral health is excluded, a management claim not independently verified in this report) are broadly consistent with this structural advantage, though they should be read as a management claim pending independent verification.

HCA's principal disadvantage is that, unlike a branded consumer or technology business, it cannot set its own prices: Medicare rates are statutory, Medicaid rates are set by individual states, and commercial rates are negotiated within a narrow band set by regional competition and payer bargaining power. This leaves HCA structurally exposed to U.S. federal and state health policy, most visibly in 2026 through the expiration of enhanced ACA premium tax credits, which management has said drove a 23.3 percent increase in uninsured admissions and an approximately \$400 million EBITDA headwind in the second quarter alone, only partly offset by higher Medicaid supplemental payments (Sourced, HCA Q2 2026 earnings call). The company's moat is assessed as Moderate: durable and structural within HCA's chosen markets, but not strong enough to insulate the business from a policy-driven demand or reimbursement shock, and not defensible against a well-capitalized nonprofit or for-profit competitor building comparable density in the same metro area.

A6. Industry Trend

The U.S. hospital industry's growth has historically tracked national health expenditure, which has grown faster than GDP for decades on an aging population and rising chronic disease prevalence; this structural demand tailwind remains intact and is the primary driver of HCA's same-facility admissions growth (same-facility admissions up 2.5 percent and emergency room visits up 3.6 percent in the second quarter of 2026, Sourced, HCA Q2 2026 earnings release). Layered on top of this structural growth is a cyclical, policy-driven disruption: the expiration of enhanced ACA premium tax credits at the end of 2025 is estimated to push 4.8 million people out of insurance coverage nationally and to cost hospitals more than \$32 billion of 2026 revenue industrywide (Sourced, Urban Institute analysis published by the Robert Wood Johnson Foundation and reported by Fierce Healthcare).

The industry is best classified as mature and structurally transforming rather than cyclical in the ordinary economic sense: care volumes are resilient to the business cycle, but reimbursement policy is not. A reversal of the current uninsured-admissions trend would most likely require Congressional action to reinstate or replace the expired premium tax credits, or a durable expansion of state-level Medicaid supplemental payment programs of the kind that has partly offset the 2026 headwind so far; absent such action, the current trend of rising uncompensated care is likely to persist through at least 2027, based on the legislative status described in the sources reviewed for this report.

A7. Management

HCA HEALTHCARE - KEY EXECUTIVES (AS OF SEPTEMBER 2026)

Name	Position	Background
Samuel N. Hazen	Chief Executive Officer and Director	Joined HCA in 1983; held multiple operating leadership roles including President of Operations and Chief Operating Officer before becoming CEO on January 1, 2019
Michael A. Marks	Chief Financial Officer and Executive Vice President	Current CFO per company disclosure as of September 2026; succeeded William B. Rutherford, who served as EVP and CFO in prior periods
Jon Mack Foster	Executive Vice President and Chief Operating Officer	HCA's three geographic operating groups report to this role
Michael R. McAlevey	Executive Vice President, Chief Legal and Administrative Officer	Oversees legal, compliance and administrative functions
Michael Schlosser, M.D.	Executive Vice President and Chief Clinical Officer	Appointed to this role effective September 1, 2026, succeeding Dr. Michael S. Cuffe
Dr. Thomas F. Frist Jr.	Founder and Chairman Emeritus	Co-founded HCA in 1968 with his father, Dr. Thomas Frist Sr.; the Frist family remained an investor group in the 2006 take-private transaction

Source: StockAnalysis.com company profile (sourced from EOD Historical Data and S&P Global Market Intelligence), accessed September 2026; Business Wire executive appointment releases, August-September 2026 (Sourced).

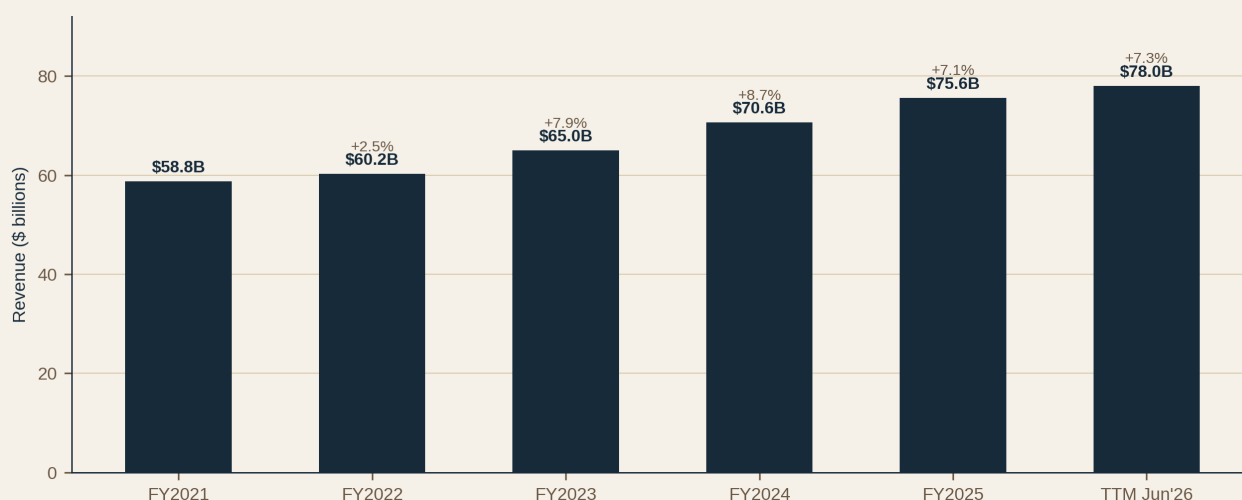
Management's capital allocation record over the past five years has favored shareholder returns over balance-sheet deleveraging: total debt rose from \$36.7 billion (FY2021) to \$48.7 billion (FY2025) while shares outstanding fell from approximately 305 million to approximately 217 million over roughly the same period, a combination that has driven strong EPS growth (EPS grew from \$21.16 in FY2021 to \$28.33 in FY2025, a period in which net income was roughly flat to slightly lower before recovering) but has also pushed book equity into negative territory. This is a deliberate, disclosed capital structure choice rather than a sign of distress, given HCA's investment-grade-adjacent credit metrics and consistent free cash flow generation, but it does mean shareholders are more exposed to a change in credit market conditions than a similarly sized company with a conventional balance sheet. Capital expenditure has grown from \$3.58 billion (FY2021) to \$4.94 billion (FY2025), broadly consistent with management's stated priority of adding capacity in growing metro markets, and the company maintains a modest but consistently growing dividend (five consecutive years of dividend growth, \$3.12 per share annualized, a 10.4 percent payout ratio) alongside the larger buyback program.

B. Evaluation

B1. Financial and Operating Metrics

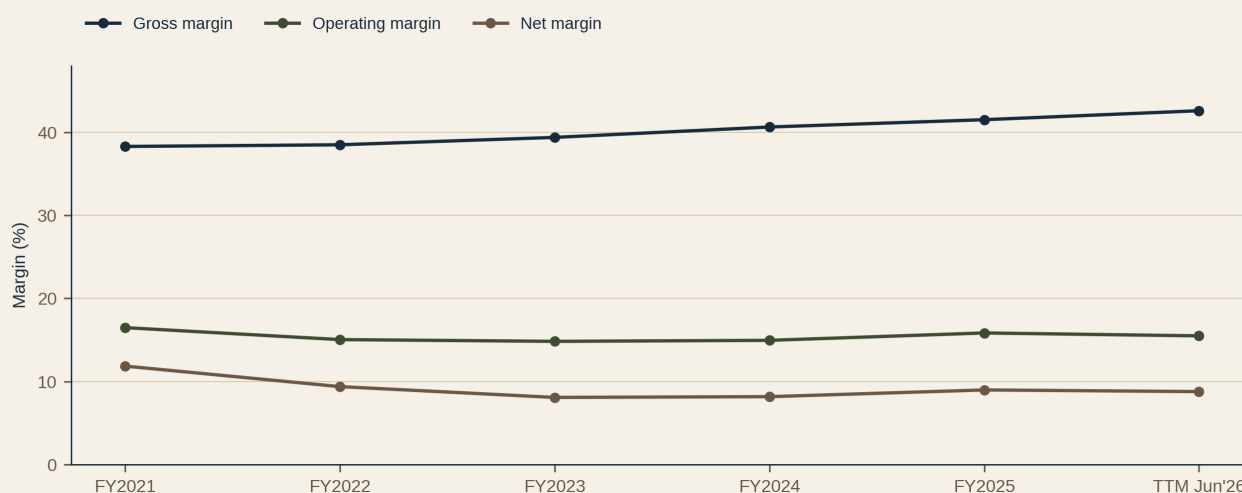
Revenue grew every year of the five-year period under review, accelerating from 2.5 percent in FY2022 to 8.7 percent in FY2024 before moderating to 7.1 percent in FY2025; the five-year (FY2021-FY2025) revenue CAGR is approximately 6.5 percent (Calculated). Gross, operating and net margins all expanded in FY2025 to their highest levels of the period shown, a reversal of the margin compression seen in FY2022-FY2023 that management attributes to a combination of volume growth, pricing and expense discipline; the trailing twelve month period shows gross and operating margin continuing to hold near FY2025 levels even as net margin has been pressured slightly by the 2026 payer-mix headwind described in Section A6.

Figure B1.1: Revenue, FY2021-TTM (Reported)



Revenue in \$ billions; growth rates shown above each bar are year-over-year. TTM period ended June 30, 2026. Source: StockAnalysis.com / S&P Global Market Intelligence (Sourced).

Figure B1.2: Gross, Operating and Net Margin, FY2021-TTM (Reported)



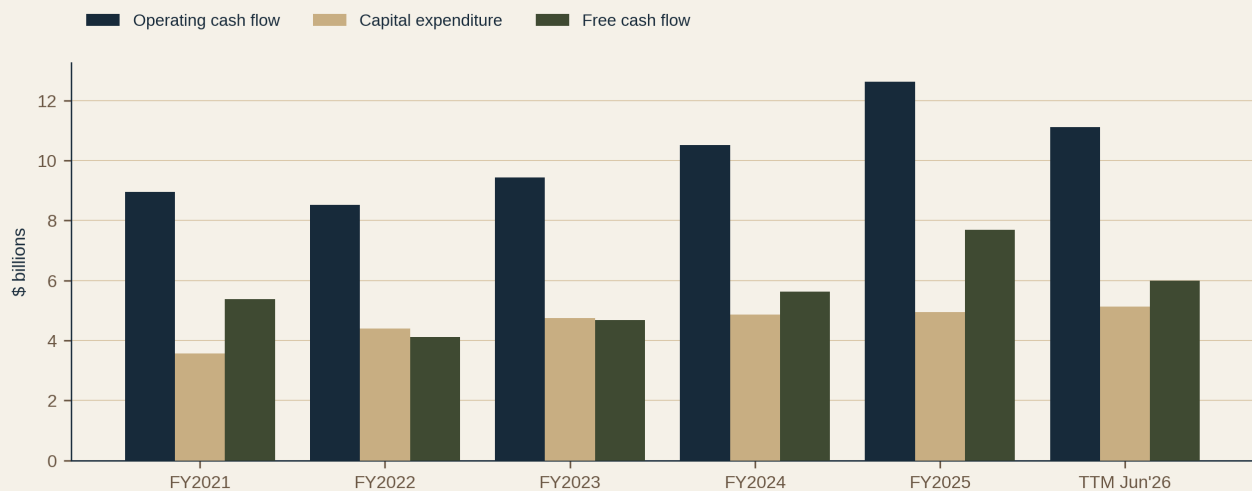
Margins calculated as the respective income statement line divided by revenue. Source: StockAnalysis.com / S&P Global Market Intelligence (Sourced).

SELECTED FINANCIAL AND OPERATING METRICS

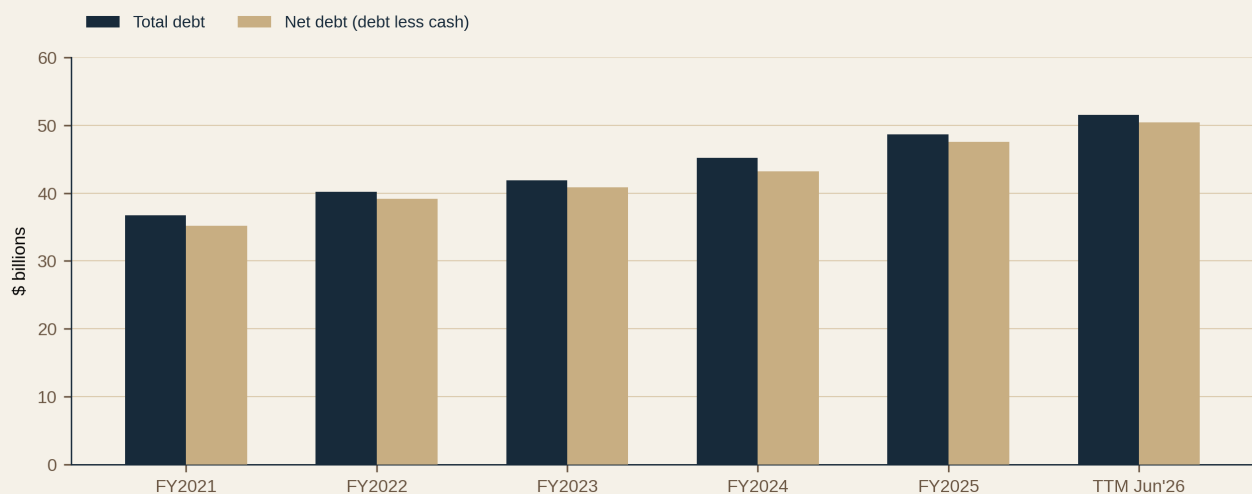
Metric	FY2023	FY2024	FY2025	TTM (Jun 2026)
Gross margin	39.37%	40.62%	41.50%	42.58%
Operating margin	14.82%	14.94%	15.83%	15.48%
Net margin	8.07%	8.16%	8.97%	8.77%
Return on assets (ROA)	Not retrieved	Not retrieved	Not retrieved	12.29%
Return on invested capital (ROIC)	Not retrieved	Not retrieved	Not retrieved	21.80%
Return on equity (ROE)	n/m	n/m	n/m	n/m (negative equity)
Asset turnover	Not retrieved	Not retrieved	Not retrieved	1.27x
Inventory turnover	Not retrieved	Not retrieved	Not retrieved	25.94x
Debt / EBITDA	Not retrieved	Not retrieved	Not retrieved	3.07x
Interest coverage (EBIT / interest)	Not retrieved	Not retrieved	Not retrieved	5.21x
FCF margin	7.21%	7.99%	10.17%	7.69%
OCF / Net income (cash conversion)	Not retrieved	Not retrieved	Not retrieved	1.63x
Capex / Revenue	7.30%	6.90%	6.54%	6.57%

"n/m" denotes not meaningful. Full-period ROA, ROIC, turnover, leverage and cash-conversion ratios were only available on a trailing-twelve-month basis from the source used (StockAnalysis.com / S&P Global Market Intelligence); FY2023-FY2025 figures for those rows are marked Not Retrieved rather than estimated. Margin and cash-flow-based rows are Calculated by the analyst from reported income statement and cash flow data where not directly disclosed as a ratio.

ROE is not a meaningful metric for HCA because book equity is negative; ROIC (21.80 percent, TTM) and ROA (12.29 percent, TTM) are the more informative profitability-on-capital measures for this company, and both indicate that HCA earns a return on invested capital well above its estimated cost of capital (discussed in Section B3). Five-year EPS growth has outpaced net income growth (EPS CAGR of approximately 7.6 percent versus a net income CAGR of roughly negative low-single-digits to flat over FY2021-FY2025, Calculated), because a falling share count, not faster profit growth, has driven much of the per-share improvement; this is an important distinction between revenue growth, business growth and EPS growth that a reader should not conflate.

Figure B1.3: Operating Cash Flow, Capital Expenditure and Free Cash Flow, FY2021-TTM

\$ billions. FCF calculated as operating cash flow less capital expenditure. Source: StockAnalysis.com / S&P Global Market Intelligence (Sourced); FCF figures Calculated by the analyst using the disclosed formula.

Figure B1.4: Total Debt and Net Debt, FY2021-TTM

\$ billions. Net debt defined as total debt less cash and short-term investments. Source: StockAnalysis.com / S&P Global Market Intelligence (Sourced).

Free cash flow grew from \$5.38 billion (FY2021) to \$7.69 billion (FY2025), a five-year CAGR of approximately 9.3 percent (Calculated) that outpaced revenue growth, reflecting margin expansion and disciplined capital spending; however, TTM free cash flow of \$6.00 billion is below the FY2025 level, consistent with the 2026 payer-mix headwind and a normalization of the unusually favorable working-capital swing that lifted FY2025 operating cash flow. Total debt has risen every year of the period shown, from \$36.7 billion (FY2021) to \$51.6 billion (TTM), while cash generation has grown more slowly in percentage terms; net income does not equal cash generation in any year shown, and the gap (illustrated by cash conversion of 1.63x TTM) is explained mainly by depreciation and amortization add-backs and the timing of receivables and payables rather than by any red flag in earnings quality.

B2. Risks

STRUCTURED RISK ASSESSMENT

Category	Risk	Probability	Impact
Business	Payer-mix deterioration: loss of ACA-exchange-subsidized patients raises the uninsured share of admissions	High (already realized in 2026)	High
Business	Geographic concentration in a subset of U.S. states leaves HCA exposed to state-specific Medicaid and regulatory decisions	Medium	Medium
Financial	Elevated leverage (Debt/EBITDA 3.07x, negative book equity) increases sensitivity to a rise in borrowing costs or a credit-market disruption	Medium	High
Financial	Working-capital volatility, illustrated by the swing between FY2025 and TTM free cash flow	Medium	Medium
Competitive	Nonprofit and other for-profit systems building comparable density in the same metro clusters	Low to Medium	Medium
Technological	Cybersecurity exposure across a large, newly-migrating electronic health record environment (MEDITECH Expanse rollout ongoing as of September 2026)	Medium	High
Regulatory	Medicare and Medicaid reimbursement policy, including the future of enhanced ACA premium tax credits and state supplemental payment programs	High	High
Macro	Labor cost inflation and nursing and allied-health staffing availability; media reporting in August 2026 described staff protests over pay and staffing levels at HCA facilities	Medium	Medium
Valuation	Multiple compression if HCA's premium versus Tenet and Universal Health Services narrows without a corresponding earnings offset	Low to Medium	Medium

Probability and impact are Reasoned, analyst judgments informed by the sourced facts cited in this report, not statistical estimates.

The three risks most capable of permanently impairing shareholder value are, in order: first, a prolonged and unreplaced loss of ACA premium tax credit subsidized patients that is not offset by Medicaid supplemental payments, which would compress margins structurally rather than temporarily; second, a rise in the cost of debt or a tightening in credit availability that raises HCA's refinancing costs against its \$51.6 billion debt load and negative book equity; and third, a sustained increase in nursing and clinical labor costs that outpaces HCA's ability to pass costs through via payer contract renegotiation.

B3. Valuation

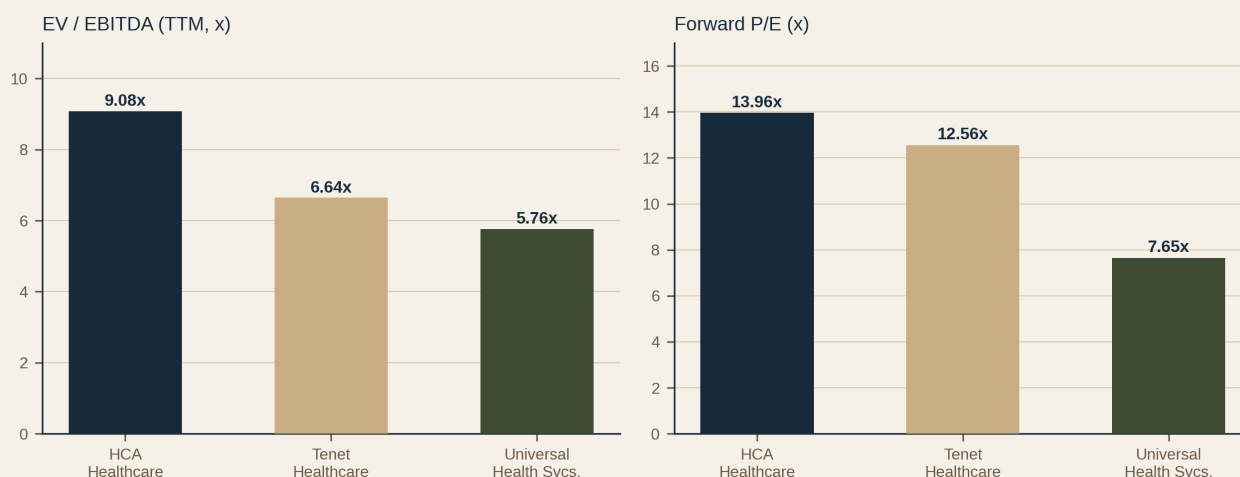
CURRENT TRADING MULTIPLES (AS OF SEPTEMBER 21, 2026 CLOSE BASIS)

Multiple	HCA (current)	HCA (FY2025)	HCA (5Y range, FY2021-FY2025)
P/E (trailing)	14.32x	15.70x	11.49x - 15.70x
Forward P/E	13.96x	15.90x	12.60x - 15.90x
P/FCF	15.44x	13.85x	13.48x - 16.44x
P/S	1.19x	1.41x	1.08x - 1.41x
EV/EBITDA	9.08x	Not retrieved	Not retrieved
EV/Sales	1.83x	Not retrieved	Not retrieved
Dividend yield	0.73%	0.62%	0.62% - 0.96%

Source: StockAnalysis.com / S&P Global Market Intelligence financial ratio history, accessed September 21, 2026 (Sourced). HCA's current P/E of 14.32x sits below its own FY2025 year-end level of 15.70x, reflecting the share price decline since the 2026 payer-mix headwind emerged, even as trailing earnings have continued to grow.

Against its closest listed peers, HCA trades at a premium on both EV/EBITDA (9.08x versus 6.64x for Tenet Healthcare and 5.76x for Universal Health Services) and forward P/E (13.96x versus 12.56x and 7.65x, respectively), shown in Figure B3.1. This premium is consistent with HCA's larger scale, more diversified metro footprint and more stable margin history rather than a valuation anomaly; Universal Health Services in particular trades at a structurally lower multiple in part because roughly a third of its earnings come from behavioral health, a segment that has historically carried lower margins and different reimbursement dynamics than acute care.

Figure B3.1: Peer Valuation Comparison (TTM basis)



Source: StockAnalysis.com / S&P Global Market Intelligence, accessed September 21, 2026 (Sourced).

Discounted Cash Flow: Illustrative Range (Reasoned)

The valuation below is a Reasoned, analyst-constructed illustration built from disclosed and estimated inputs; it is not a price target and should not be read as a precise intrinsic value. A single-stage, five-year explicit free cash flow projection is discounted at a range of weighted average cost of capital (WACC) assumptions, followed by a Gordon growth terminal

value. The base free cash flow is TTM free cash flow of \$6.00 billion (Sourced), grown at 4 percent per year for five years (Reasoned, informed by the 3-year consensus revenue growth forecast of 4.22 percent sourced from StockAnalysis.com analyst estimates), discounted at WACC assumptions of 7 percent, 8 percent and 9 percent (the 8 percent case sits close to both the third-party-sourced WACC of 7.93 percent and the analyst's own CAPM cross-check, described below), against terminal growth assumptions of 1.5 percent, 2.25 percent and 3.0 percent. Net debt of \$50.42 billion (TTM, Sourced) is subtracted from enterprise value, and the result is divided by 216.5 million shares outstanding (Sourced) to arrive at an illustrative equity value per share.

As a cross-check on WACC: using a risk-free rate of 5.00 percent (10-year U.S. Treasury yield, Sourced, TradingEconomics, September 18, 2026), an assumed equity risk premium of 4.5 percent and HCA's beta of 1.11 (Sourced), CAPM implies a cost of equity of approximately 10.0 percent; backing out an implied pre-tax cost of debt from TTM interest coverage (EBIT of \$12.07 billion divided by 5.21x coverage implies interest expense of approximately \$2.32 billion against \$51.6 billion of total debt, or roughly 4.5 percent pre-tax, 3.6 percent after HCA's 20.4 percent effective tax rate) and weighting by the current market values of equity (\$92.60 billion) and debt (\$51.55 billion) produces a WACC of approximately 7.7 percent (Calculated), consistent with the 7 to 9 percent range used in the sensitivity table below.

ILLUSTRATIVE EQUITY VALUE PER SHARE: WACC X TERMINAL GROWTH SENSITIVITY (REASONED)

	Terminal growth 1.5%	Terminal growth 2.25%	Terminal growth 3.0%
WACC 7.0%	\$338	\$412	\$514
WACC 8.0%	\$249	\$299	\$364
WACC 9.0%	\$184	\$220	\$264

Reasoned, analyst-constructed sensitivity table; all inputs stated above. Because HCA's capital structure carries substantial net debt relative to enterprise value, the residual equity value is highly sensitive to small changes in WACC and terminal growth; this is a structural feature of HCA's leverage, not a modeling artifact.

The model's central case (WACC 8.0 percent, terminal growth 2.25 percent) implies an equity value of approximately \$299 per share, below the September 18, 2026 closing price of \$427.71. The market price sits closer to the upper end of the illustrative range shown, which is consistent with two, non-mutually-exclusive explanations: the market may be pricing free cash flow growth above the 4 percent per year used in the base case (plausible given FY2025 FCF growth of over 30 percent, even if that pace is unlikely to repeat), or the market may be applying a lower effective discount rate than the 7 to 9 percent range used here. This gap between the DCF central case and the market price is a data point for the reader to weigh, not a conclusion that the stock is overvalued; the DCF is one valuation lens among several presented in this report, and the multiples-based comparison in Section B3 shows HCA trading in line with, not above, its own recent historical range.

What Is Priced In

At current multiples, the market appears to be pricing continued mid-single-digit revenue growth, stable-to-improving operating margins, and a successful offset of the 2026 payer-mix headwind through Medicaid supplemental payments and continued volume growth, consistent with the analyst three-year consensus of 4.22 percent revenue growth and 7.55 percent EPS growth (Sourced, StockAnalysis.com analyst forecast aggregation). These expectations are assessed as Reasonable rather than Conservative or Extremely Optimistic: they are broadly consistent with HCA's own five-year growth history and do not appear to require a resolution of the ACA subsidy uncertainty in HCA's favor, though a favorable resolution would represent upside to consensus and an unfavorable one (for example, a further reduction in Medicaid supplemental payments) would represent downside.

B4. Capital Market and Stock Analysis

PRICE PERFORMANCE (PERIOD RETURNS TO SEPTEMBER 18, 2026 CLOSE OF \$427.71)

Period	Return
1 day	-0.31%
1 week	+0.18%
1 month	+3.88%
3 months (Jun 22 - Sep 18, 2026)	+13.45%
52 weeks	+7.09%
6 months, YTD, 3 years, 5 years	Not retrieved in this report

1-day through 3-month returns Calculated by the analyst from daily closing prices sourced from StockAnalysis.com. The 52-week return is Sourced directly (StockAnalysis.com statistics page, accessed September 21, 2026). Longer-dated returns were not retrieved from the sources reviewed for this report and are not estimated.

Within the trailing three-month window shown on the cover chart, HCA fell sharply (-6.95 percent in a single session) on July 14, 2026, coinciding with the company's preview of second-quarter results and the emerging visibility of the ACA subsidy-driven payer-mix headwind; the stock's maximum peak-to-trough drawdown inside this window was approximately -14.1 percent (from the July 7 close of \$423.11 to the July 14 close of \$363.60, Calculated). The stock has since recovered most of that decline, closing at \$427.71 on September 18, 2026, roughly in line with its level in early July before the drawdown.

TECHNICAL AND VOLATILITY SNAPSHOT (AS OF SEPTEMBER 21, 2026)

Indicator	Value
50-day moving average	\$406.30
200-day moving average	\$448.69
Relative Strength Index (14-day)	58.56
Beta (5-year, versus broad market)	1.11
Average daily volume (20-day)	1.54 million shares
Short interest (% of float)	4.25%

Source: StockAnalysis.com statistics page (Sourced from S&P Global Market Intelligence and CBOE data), accessed September 21, 2026.

HCA trades above its 50-day moving average but below its 200-day moving average, a technical structure consistent with a stock that fell sharply mid-period on the July 2026 guidance reaction and has since recovered without yet reclaiming its longer-term trend line; an RSI of 58.56 indicates the stock is neither overbought nor oversold at the report date. This technical positioning should be read separately from the fundamental valuation discussion in Section B3: a recovering short-term trend does not, on its own, indicate whether the shares are fundamentally cheap or expensive, and readers should weigh the two lenses independently rather than combining them into a single signal.

C. Financial Statements

The following analysis covers HCA's five most recently reported fiscal years (FY2021-FY2025, each ended December 31) plus the trailing twelve months ended June 30, 2026. HCA reports under U.S. GAAP. All figures are Sourced from HCA Healthcare's Form 10-K and Form 10-Q filings as aggregated by S&P Global Market Intelligence and presented by StockAnalysis.com, unless marked Calculated (a ratio derived by the analyst from two or more reported figures) or Reasoned (an analyst judgment or estimate).

C1. Income Statement

CONSOLIDATED INCOME STATEMENT SUMMARY (\$ MILLIONS EXCEPT PER-SHARE DATA)

Line Item	FY2021	FY2022	FY2023	FY2024	FY2025	TTM Jun'26
Revenue	58,752	60,233	64,968	70,603	75,600	78,013
Revenue growth	14.0%	2.5%	7.9%	8.7%	7.1%	7.3%
Gross profit	22,492	23,177	25,579	28,678	31,374	33,218
Operating income (EBIT)	9,678	9,054	9,626	10,547	11,965	12,073
Pretax income	9,838	8,576	7,706	8,522	9,836	9,869
Net income (attributable to HCA)	6,956	5,643	5,242	5,760	6,784	6,840
Diluted EPS	\$21.16	\$19.15	\$18.97	\$22.00	\$28.33	\$29.87
Effective tax rate	Not retrieved	Not retrieved	Not retrieved	Not retrieved	Not retrieved	20.44%

Pretax income Calculated by the analyst as operating income less an implied net interest and other expense figure consistent with reported pretax margin; all other rows Sourced (StockAnalysis.com / S&P Global Market Intelligence). R&D expense is not a material or separately disclosed line item for a hospital operator and is omitted as not economically meaningful for this business.

Revenue growth has been driven primarily by volume (same-facility admissions and equivalent admissions growth, reported quarterly by management) and by favorable pricing and payer mix in most years, with bolt-on acquisitions (such as the September 2026 purchase of the College of Health Care Professions) a secondary, smaller contributor; management has not disclosed a currency effect, as HCA's operations are almost entirely U.S.-dollar-denominated with a small U.K. presence. Operating leverage has been positive in four of the five years shown, with operating margin expanding from 14.82 percent (FY2023) to 15.83 percent (FY2025) as revenue growth outpaced the growth of salaries, wages and benefits and supply costs; margins are assessed as expanding over the period as a whole, though the TTM period shows a slight moderation (15.48 percent) consistent with the 2026 payer-mix headwind discussed in Section A6.

C2. Balance Sheet

CONSOLIDATED BALANCE SHEET SUMMARY (\$ MILLIONS)

Line Item	FY2021	FY2022	FY2023	FY2024	FY2025	TTM Jun'26
Cash and short-term investments	1,554	1,000	1,022	2,021	1,143	1,130
Accounts receivable	8,106	8,903	9,966	10,796	10,876	12,281
Total current assets	13,542	13,643	14,927	16,414	15,783	17,190
Property, plant and equipment	26,176	27,640	29,922	31,545	33,271	33,925
Goodwill	9,137	9,294	9,606	9,770	9,982	Not retrieved
Total assets	50,742	52,438	56,211	59,513	60,720	63,250
Total current liabilities	9,582	9,902	12,655	15,177	16,350	17,312
Long-term debt	33,765	37,135	36,628	37,709	40,978	43,454
Total liabilities	49,253	52,511	55,151	58,958	63,491	66,459
Total shareholders' equity (incl. minority interest)	1,489	(73)	1,060	555	(2,771)	(3,209)
Total debt	36,734	40,200	41,859	45,237	48,697	51,552
Net debt	35,180	39,200	40,837	43,216	47,554	50,422

Source: StockAnalysis.com / S&P Global Market Intelligence, standardized balance sheet, accessed September 2026 (Sourced). Figures in parentheses are negative.

BALANCE SHEET RATIOS

Ratio	FY2023	FY2024	FY2025
Current ratio	1.18x	1.08x	0.97x
Working capital (\$m)	2,272	1,237	(567)
Equity ratio (equity / total assets)	1.9%	0.9%	(4.6%)
Debt / EBITDA	Not retrieved	Not retrieved	Not retrieved

Current ratio and working capital Calculated by the analyst from the balance sheet data above; equity ratio Calculated as total equity divided by total assets. Debt/EBITDA was only available on a TTM basis (3.07x, Sourced) from the source used in this report and is not estimated for individual fiscal years.

HCA's balance sheet shows two structural features that a reader should not mistake for financial distress: a working capital position that turned modestly negative in FY2025 (current ratio of 0.97x), and total equity that has been negative in three of the past two years shown here, turning most sharply negative in FY2025 and remaining negative on a TTM basis. Both are consequences of a sustained, debt-funded share repurchase program rather than operating losses; HCA has been profitable in every year shown, and its Altman Z-Score of 2.64 (Sourced, TTM) sits in the "grey zone" between clear financial health and distress typically associated with such a score, which is itself an artifact of the negative-equity, high-leverage capital structure rather than deteriorating operating performance. Goodwill has grown steadily but modestly (from \$9.1 billion to \$10.0 billion over the period shown) relative to total assets, and there is no evidence in the data reviewed of goodwill

impairment risk or unusual receivables or inventory buildup; accounts receivable growth (from \$8.1 billion to \$10.9-12.3 billion) has broadly tracked revenue growth rather than outpacing it.

C3. Cash Flow Statement

CONSOLIDATED CASH FLOW SUMMARY (\$ MILLIONS)

Line Item	FY2021	FY2022	FY2023	FY2024	FY2025	TTM Jun'26
Operating cash flow	8,959	8,522	9,431	10,514	12,636	11,124
Capital expenditure	(3,577)	(4,395)	(4,744)	(4,875)	(4,944)	(5,127)
Free cash flow	5,382	4,127	4,687	5,639	7,692	5,997
FCF growth	(15.9%)	(23.3%)	13.6%	20.3%	36.4%	(17.8%)

Source: StockAnalysis.com / S&P Global Market Intelligence (Sourced). Figures in parentheses are negative or, for growth rates, declines.

Free cash flow equals operating cash flow less capital expenditure in every year shown. HCA has generated positive free cash flow throughout the five-year period, with FY2025 the strongest year on both an absolute (\$7.69 billion) and FCF-margin (10.17 percent) basis, aided by working-capital timing that has partially reversed in the trailing twelve months (TTM FCF of \$6.00 billion, an 17.8 percent decline from the FY2025 level). Net income has not equaled cash generation in any year: operating cash flow has exceeded net income throughout the period (TTM cash conversion of 1.63x), consistent with the add-back of substantial depreciation and amortization on a large, growing fixed-asset base, and there is no indication in the data reviewed that reported earnings are being inflated relative to cash generation; if anything, the relationship between earnings and cash flow supports the quality of HCA's reported net income.

On financing activities, HCA has consistently used free cash flow plus incremental borrowing to fund share repurchases (buyback yield of 8.97 percent, TTM) and a modest but growing dividend (\$3.12 per share annualized, a 10.4 percent payout ratio, TTM), rather than to reduce debt; net debt has risen in every year shown. This is a explicit, disclosed capital allocation choice that has driven strong shareholder returns through EPS accretion but leaves less balance-sheet flexibility than a peer prioritizing deleveraging.

C4. Forecasting and Projections

Revenue modeling for HCA is best approached as a function of same-facility volume growth (admissions and equivalent admissions), payer mix, and pricing, rather than a single blended growth rate; management's own FY2026 guidance, issued alongside second-quarter results, calls for full-year revenue of \$77.0 to \$79.5 billion, Adjusted EBITDA of \$15.4 to \$16.1 billion, net income of \$6.3 to \$6.7 billion and diluted EPS of \$28.70 to \$30.50 (Sourced, HCA Q2 2026 earnings release). The midpoint of this guidance (\$78.25 billion revenue) implies growth of approximately 3.5 percent over FY2025, slower than the 7.1 percent recorded in FY2025, reflecting management's own expectation that the payer-mix headwind will outweigh continued volume growth over the balance of 2026.

SCENARIO ANALYSIS: FY2026 REVENUE AND MARGIN (REASONED, INFORMED BY MANAGEMENT GUIDANCE AND ANALYST CONSENSUS)

Scenario	FY2026 Revenue	FY2026 Adj. EBITDA	Key Driver
Bear case	~\$77.0B (guidance low end)	~\$15.4B	Uninsured admissions remain elevated; supplemental Medicaid payments only partly offset the exchange-subsidy loss
Base case (management guidance midpoint)	~\$78.25B	~\$15.75B	Volume growth continues near 2-3%; payer-mix headwind stabilizes near current levels
Bull case	~\$79.5B (guidance high end)	~\$16.1B	Supplemental payment programs expand further and/or a partial restoration of exchange subsidies reduces uninsured admissions

Revenue and Adjusted EBITDA ranges Sourced directly from HCA Healthcare's FY2026 guidance (Q2 2026 earnings release, July 14, 2026); scenario labels and narrative drivers are Reasoned by the analyst, not management's own scenario language.

Working capital and capital expenditure are expected to remain a modest but rising share of revenue: capital expenditure has grown from 6.1 percent of revenue (FY2021) to a range of 6.5 to 7.3 percent over FY2023-TTM, reflecting continued new-capacity investment in growing metro markets; the company has not signaled a change to this intensity. The single variable most likely to move actual results away from the base case over the next 12 months is the trajectory of state-level Medicaid supplemental payment programs, which management has cited directly as the primary offset to lost exchange-subsidized volume (Sourced, HCA Q2 2026 earnings call); a further expansion or contraction of these programs would have a larger near-term effect on HCA's reported EBITDA than any single volume or pricing variable.

C5. Discounted Cash Flow Model

The DCF mechanics, full assumption set and sensitivity table are presented in Section B3 to keep the valuation discussion in one place; they are summarized here for completeness, as required by the standard structure of this report.

REVENUE ASSUMPTION	Base FCF of \$6.00 billion (TTM, Sourced) rather than a full revenue build; implicitly consistent with 3-year consensus revenue growth of 4.22% (Sourced)
FCF GROWTH ASSUMPTION	4% per year for 5 years (Reasoned)
TAX ASSUMPTION	20.44% effective tax rate (TTM, Sourced) embedded in the after-tax cost of debt calculation
WACC	7.0% to 9.0% range tested (third-party-sourced WACC: 7.93%; analyst CAPM cross-check: approximately 7.7%, Calculated)
TERMINAL GROWTH	1.5% to 3.0% range tested (Reasoned)
NET DEBT	\$50.42 billion (TTM, Sourced)
SHARES OUTSTANDING	216.5 million (Sourced)
RESULTING RANGE	Approximately \$184 to \$514 per share across the 3x3 sensitivity grid; central case (WACC 8.0%, terminal growth 2.25%) of approximately \$299 per share

As stated in Section B3, this range is an illustrative construct, not a price target, and the reader should note that HCA's negative book equity and elevated leverage make the equity residual unusually sensitive to the WACC and terminal growth assumptions chosen.

C6. Leveraged Buyout Model

A fresh leveraged buyout model is not constructed for this report. At a \$92.60 billion market capitalization and \$143.02 billion enterprise value, HCA is too large relative to current private-equity fund sizes and leveraged-loan and high-yield market capacity for a going-private transaction to be a realistic near-term scenario, and HCA already carries Debt/EBITDA of 3.07x, limiting the incremental leverage a financial sponsor could add without materially raising the cost of debt. HCA's own history is nonetheless directly relevant context: the company was taken private in November 2006 by Kohlberg Kravis Roberts, Bain Capital and Merrill Lynch Global Private Equity, together with the Frist family, in a transaction valued at approximately \$33 billion, at the time the largest leveraged buyout in history (Sourced, contemporaneous reporting including insideARM and PitchBook), before returning to public markets in March 2011 through what was then the largest private-equity-backed IPO in U.S. history, pricing at \$30 per share and raising approximately \$3.8 billion (Sourced). That transaction demonstrates that HCA's asset base and cash flow profile have historically supported very high leverage; the present-day capital structure (Debt/EBITDA of 3.07x) is materially more conservative than the leverage carried immediately after the 2006 buyout.

D. Investment Thesis

Bull Case

HCA outperforms current expectations if same-facility volume growth (admissions up 2.5 percent and emergency room visits up 3.6 percent in the most recent quarter) continues at or above its recent pace while Medicaid supplemental payment programs expand further, offsetting more of the lost ACA-exchange-subsidized volume than the market currently assumes; combined with continued operating leverage and an unchanged capital return program, this could push margins back toward or above the FY2025 peak (15.83 percent operating margin) and support renewed multiple expansion if HCA's premium over Tenet Healthcare and Universal Health Services widens on the back of demonstrated resilience.

Bear Case

HCA underperforms if the 2026 payer-mix headwind proves more persistent or severe than management's guidance assumes, if state-level Medicaid supplemental payment programs are curtailed rather than expanded, or if labor cost inflation (already a visible point of tension, per the August 2026 reporting on nurse and staff pay protests) accelerates faster than pricing can offset. Given HCA's negative book equity and 3.07x Debt/EBITDA, a simultaneous earnings disappointment and a rise in borrowing costs would compress both the earnings base and the multiple applied to it, a combination the DCF sensitivity table in Section B3 shows can move the illustrative equity value by more than 2x between the bear and bull ends of the range tested. Labor relations tension was visible in media reporting during August 2026 around pay and staffing levels at HCA facilities.

Base Case

The most reasonable near-term scenario, consistent with management's own FY2026 guidance, is revenue in the \$77 to \$79.5 billion range (Sourced), Adjusted EBITDA of \$15.4 to \$16.1 billion, continued but moderated volume growth, and a payer-mix headwind that is materially offset, though not fully neutralized, by Medicaid supplemental payments and continued share repurchases. Under this base case, margins hold roughly at current TTM levels rather than expanding meaningfully, and the current premium valuation versus hospital-sector peers is sustained rather than re-rated in either direction.

Catalysts

The next quarterly earnings release (expected October 23, 2026, Sourced) will be the most immediate catalyst, given the market's evident sensitivity to payer-mix commentary since the July 2026 guidance preview. Other potential catalysts include further bolt-on acquisitions in the workforce-pipeline or ambulatory space (following the September 2026 College of Health Care Professions acquisition), continued rollout of the MEDITECH Expanse electronic health record platform across additional divisions, any Congressional action on the expired enhanced ACA premium tax credits, and the pace of continued share repurchases.

Things to Monitor

MONITORING LIST

Metric or Event	Why It Matters	Positive Signal	Negative Signal
Uninsured admissions growth	Direct measure of the ACA-subsidy payer-mix headwind	Growth rate decelerates from the 23.3% pace reported in Q2 2026	Uninsured admissions growth accelerates further
State Medicaid supplemental payment programs	Primary disclosed offset to the payer-mix headwind	States expand or extend supplemental payment programs	Programs are curtailed or reduced
Same-facility admissions and equivalent admissions	Core organic volume growth driver	Growth sustained at or above 2-3%	Growth decelerates toward flat or negative
Net debt and Debt/EBITDA	Leverage headroom given negative book equity	Debt/EBITDA holds near or below 3.0x	Debt/EBITDA rises materially above 3.0x
Labor cost per adjusted admission	Largest single operating cost category	Labor cost growth remains below revenue growth	Labor cost growth outpaces revenue growth
Share repurchase pace	Primary driver of EPS growth in excess of net income growth	Buybacks continue at a similar or greater pace	Buybacks are curtailed to preserve balance-sheet flexibility

E. Final Investment Framework

BUSINESS QUALITY	High: the largest for-profit hospital operator in the United States by scale, bed count and net patient revenue, with a decentralized, market-density operating model that has produced consistent volume and revenue growth.
FINANCIAL QUALITY	Mixed: strong, growing operating cash flow and high cash conversion, offset by a deliberately leveraged, negative-book-equity capital structure that reduces balance-sheet flexibility.
COMPETITIVE POSITION	Market leader by scale versus Tenet Healthcare, Universal Health Services and Community Health Systems, with a Moderate, structural but not unassailable moat built on local market density.
MANAGEMENT	Long-tenured, operationally experienced leadership (CEO Samuel Hazen has 42 years at HCA) with a clear, consistently executed capital allocation priority favoring shareholder returns over deleveraging.
INDUSTRY	Mature and structurally transforming: durable demographic demand growth is currently overshadowed by a policy-driven, 2026 payer-mix shock tied to the expiration of enhanced ACA premium tax credits.
VALUATION	Trading at a premium to hospital-sector peers on EV/EBITDA (9.08x versus 6.64x and 5.76x) and roughly in line with its own five-year historical P/E range; a Reasoned DCF central case of approximately \$299 per share sits below the current market price of \$427.71, a gap the reader should weigh alongside the multiples-based comparison.
TECHNICAL AND MARKET STRUCTURE	Recovering from a sharp mid-2026 drawdown (-14.1% peak to trough within the trailing three-month window), trading above its 50-day but below its 200-day moving average, with an RSI near neutral and an analyst consensus of Buy.
PRINCIPAL RISK	A prolonged, unreplaced loss of ACA-exchange-subsidized patient volume that Medicaid supplemental payment programs do not fully offset.
PRINCIPAL CATALYST	Third-quarter 2026 earnings (expected October 23, 2026), the market's next direct readout on the payer-mix trend.
KEY VARIABLE	The trajectory of state-level Medicaid supplemental payment programs relative to the continued loss of ACA-exchange-subsidized patients.
THESIS INVALIDATION	The investment thesis would become materially weaker if uninsured admissions growth accelerates beyond the 23.3% pace reported in the second quarter of 2026 without a corresponding expansion of Medicaid supplemental payments, or if Debt/EBITDA rises sustainably above approximately 3.5x amid a rising interest-rate environment.

F. Data Table

STANDARDIZED COMPANY SNAPSHOT

Category	Metric	Current	Prior Year	3Y / 5Y Trend
Market	Market capitalization	\$92.60B	Not retrieved	Not retrieved for prior periods
Market	Enterprise value	\$143.02B	Not retrieved	Not retrieved for prior periods
Market	Share price	\$427.71	~\$399 (Calculated from 52-wk change)	+7.1% over the trailing 52 weeks
Growth	Revenue growth (YoY)	7.3% (TTM)	8.7% (FY2024)	5Y range 2.5% to 14.0%; FY21-25 CAGR ~6.5%
Profitability	Gross margin	42.58%	40.62% (FY2024)	Expanding every year, FY2021-TTM
Profitability	Operating margin	15.48%	14.94% (FY2024)	5Y range 14.82% to 16.47%
Profitability	Net margin	8.77%	8.16% (FY2024)	5Y range 8.07% to 11.84%; below FY2021 peak
Profitability	ROE	n/m	n/m	Not meaningful; book equity negative since FY2025
Profitability	ROIC	21.80%	Not retrieved	Not retrieved for prior periods
Cash Flow	Operating cash flow	\$11.12B (TTM)	\$10.51B (FY2024)	Rose every year FY2021-FY2025 (peak \$12.64B)
Cash Flow	Free cash flow	\$6.00B (TTM)	\$5.64B (FY2024)	FY2025 peak \$7.69B; FY21-25 CAGR ~9.3%
Leverage	Net debt / EBITDA	3.07x	Not retrieved	Net debt rose every year, \$35.2B to \$50.4B
Valuation	P/E (trailing)	14.32x	13.20x (FY2024 year-end)	5Y range 11.49x to 15.70x
Valuation	EV/EBITDA	9.08x	Not retrieved	Not retrieved for prior periods
Valuation	P/FCF	15.44x	13.48x (FY2024 year-end)	5Y range 13.48x to 16.44x
Market	Beta (5-year)	1.11	n/a	n/a
Market	Annualized volatility	36.0% (Calculated, trailing 3-month sample)	Not retrieved	Not retrieved for prior periods
Market	Maximum drawdown	-14.1% (trailing 3-month window)	Not retrieved	Not retrieved for prior periods

"n/m" denotes not meaningful; "n/a" denotes not applicable. Sourced figures are from StockAnalysis.com / S&P Global Market Intelligence, accessed September 18-21, 2026. Calculated figures are derived by the analyst as described in the relevant section of this report and in the footnotes above. Prior-year and multi-year trend figures not directly available from the sources reviewed are marked Not Retrieved rather than estimated.

G. Sources and Methodology

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Calculations

All figures explicitly marked "Calculated" throughout this report (including revenue, EPS and free cash flow CAGRs; pretax income by year; cash conversion; capex-to-revenue; current and quick ratios; the CAPM-based WACC cross-check; the DCF sensitivity table; trailing three-month annualized volatility and maximum drawdown; and short-dated price returns) were derived by the analyst from the Sourced figures listed above, using the formulas stated in the relevant section of this report. Figures explicitly marked "Reasoned" are analyst judgments or illustrative assumptions, not sourced facts, and are labeled as such at each use.

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